

****HOSPITAL DISCHARGE SUMMARY****

****PATIENT DEMOGRAPHICS****

Name: Patient 31

Age: 68 years old | Sex: Male

MRN: 000-TEST-31

Date of Admission: 06/12/2026

Date of Discharge: 06/15/2026

Attending Physician: Dr. R. Mendez, MD (Orthopedic Surgery)

PRIMARY DIAGNOSIS & COMORBIDITIES

Patient 31 admitted for elective left total hip arthroplasty (THA) secondary to severe osteoarthritis with mechanical symptoms and functional limitation refractory to conservative management. Cemented prosthesis placed (Zimmer® Biomet cement fixation). Relevant comorbidities include hypertension (on lisinopril) and chronic kidney disease stage 3 (baseline Cr 1.7 mg/dL, eGFR 38 mL/min).

BRIEF HOSPITAL COURSE

68 y/o male presented 06/12/2026 for elective left THA. Preoperative labs and imaging unremarkable; EKG showed normal sinus rhythm. Patient underwent cemented left THA on 06/12/2026 without intraoperative complications. Estimated blood loss 450 mL; no transfusion required. Post-op course uncomplicated:

- POD #1: Vitals stable, SpO2 94–98% on room air. Pain controlled with multimodal regimen (see Medications). Foley catheter d/c'd. Patient ambulating with walker; PT/OT consulted.

- POD #2: Incision clean, dry, intact (CDI). No erythema or drainage. Prophylactic DVT measures continued. Patient tolerating regular diet.
- POD #3: Continued progress with PT; patient performing straight-leg raises and quad sets. Discharged to home with home health PT/OT.

DISCHARGE MEDICATIONS

Medication	Dose	Route	Frequency	Duration
Acetaminophen	650 mg	PO	q.6h. PRN pain	2 weeks then reassess
Oxycodone	5 mg	PO	q.4–6h. PRN moderate–severe pain	7 days; then d/c if tolerated
Enoxaparin (Lovenox)	40 mg	SC	q.d.	10 days (DVT prophylaxis)
Lisinopril	10 mg	PO	q.d.	Continue indefinitely
Docusate sodium	100 mg	PO	BID	2 weeks (constipation prevention)
Ibuprofen	400 mg	PO	q.6–8h. PRN pain/inflammation	2 weeks max (CKD consideration)

ACTIVITY & WEIGHT-BEARING STATUS

****Partial Weight-Bearing (PWB) left lower extremity**** — 50% weight-bearing as tolerated with walker. Progress to WBAT (weight-bearing as tolerated) only after clearance from orthopedic surgeon at 4-week post-op visit.

****HIP PRECAUTIONS — STRICT ADHERENCE MANDATORY:****

- No hip flexion >90 degrees (use elevated toilet seat; no low couches/chairs)
- No internal rotation or adduction of left hip

- No crossing legs
- Use abductor pillow between legs while supine and side-lying
- Avoid bending forward to pick up objects

WOUND CARE & FOLLOW-UP APPOINTMENTS

Incision: staples in place; keep dry and clean. Dressing changes by home health RN. Patient instructed to report any signs of infection (